The Birth and Development of Medicaid

Medicaid is one of America's most vital healthcare programs, and it uses taxpayer funding to help support low income families. Currently, it contributes to nearly 1/5th of healthcare spending and supports about 80 million low-income families in America. The program, however, required modifications in who qualified and what benefits were covered to strengthen it to what it is today.

Medicaid was signed into law by former President Lyndon B. Johnson in 1965 as part of Title XIX of the Social Security Amendments, and it provided health insurance for people who had a limited income that barred them from receiving proper healthcare treatment. This early version of Medicaid was not a perfect solution to address lack of healthcare access as Medicaid coverage was… tied to cash assistance—either Aid to Families with Dependent Children (AFDC) or Federal Supplemental Security Income (SSI)" (Burns, Alice, et. al). This meant that families who were unable to receive cash assistance were also unable to receive Medicaid benefits; as a result, many people who needed healthcare were denied it. Furthermore, it was only in 1982 that Arizona became the last state to implement the program. This meant that between 1966 (when the program was actually implemented) and 1982, not all Americans who could potentially have been covered by Medicaid received the help they needed.

In the 1980s and 1990s, the scope of Medicaid expanded to include a wider variety of individuals: pregnant women, children, people with disabilities, and a larger low-income population. Cash assistance was no longer an eligibility requirement for Medicaid, and the Children's Health Insurance Program (CHIP) allowed low-income children to be covered even if their family's income was too high for Medicaid. Furthermore, the quality of care to those insured by Medicaid increased through developments like the Omnibus Budget Reconciliation Act (OBRA) and its expansions which provided for health coverage plans, protection in long-term care facilities, and coverage of required medical services for youths under the age of 21. Lastly, prescription drug prices decreased for Medicaid users.

The Affordable Healthcare Act (ACA) was the most recent major expansion of Medicaid for both children and adults. While the expansion in regard to adults was deemed optional in the 2012 Supreme

Court ruling, only 10 states chose not to adopt the expansion. The other 41 states (including the District of Columbia) adopted and implemented Medicaid for most Americans up to 138% of the Federal Poverty Level (FPL), giving Medicaid access to adults without dependent children. Furthermore, all states were prevented from lowering the Medicaid eligibility threshold and were forced to keep eligibility at a minimum at 138% FPL regardless of whether they chose to adopt the expansion or not.

Medicaid today is quite different from when it was signed by Johnson. It experienced expansions in not only who was covered, but what was covered by insurance. However, as we have seen historically, Medicaid and its expansions have not gone unchallenged. Policies like the One Big Beautiful Bill Act (OBBBA) have decreased healthcare coverage by adding work requirements and cutting funding. As American citizens and health advocates, it is important to know how and why programs like Medicaid progressed in order to understand what is at stake if we lose them.

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